CREDIT MEMO · BUSINESS OVERVIEW

HCA Healthcare, Inc.

Reporting period: 2025-12-31 · Evidence cutoff: 2026-03-31T00:00:00+00:00

Business Overview completeness: **Incomplete** · Recorded outcome: **human review required** · Current human review: **Pending**

01 Corporate Structure & Debt Obligations

Corporate Structure

Corporate structure is not available for this run.

> structure: Corporate onboarding JSON unavailable

Debt Obligations

Recorded debt figures. Complete instrument schedules, guarantees, security and creditor-priority detail follow on subsequent pages.

Recorded measure	Value
Direct operating company secured (US$ m)	585.0
Senior unsecured guaranteed (US$ m)	45907.0
Unguaranteed holding company (US$ m)	0.0
Consolidated indebtedness (US$ m)	46492.0
Structural subordination drag ratio	0.0126

> Debt population is preserved as supplied; recorded validation does not establish completeness beyond source scope.

Complete Debt Obligations

> structure: Corporate onboarding JSON unavailable

Senior notes

Senior note / series	Legal borrower	Principal (US$ m)	Coupon / rate	Maturity	Security	Guarantors / credit support	Priority tier
Senior Unsecured Notes	HCA Inc.	43700.0	5.1% (weighted average)	2095	Unsecured	HCA Healthcare, Inc. and subsidiaries	Tier 2 · Senior unsecured guaranteed

Non-note obligations and facilities

Obligation / facility	Legal borrower	Drawn (US$ m)	Facility limit (US$ m)	Interest terms	Maturity	Security	Guarantors / credit support	Priority tier
Commercial Paper	HCA Inc.	2207.0	4000.0	4.3% (weighted average)	Variable (max 397 days)	Unsecured	HCA Healthcare, Inc. guarantee implied	Tier 2 · Senior unsecured guaranteed
Finance Leases	HCA Inc.	585.0	Not recorded	Effective rate 4.6%	Varies	Secured	Asset-backed	Tier 1 · Direct operating company secured

Creditor priority and reconciliation

Direct operating company secured (US$ m)	585.0
Senior unsecured guaranteed (US$ m)	45907.0
Unguaranteed holding company (US$ m)	0.0
Consolidated indebtedness (US$ m)	46492.0
Structural subordination drag ratio	0.0126
Prospective secured loan placement	TIER_1

Prospective unsecured loan placement	TIER_2

> Debt population is preserved as supplied; recorded validation does not establish completeness beyond source scope.

02 Management

Management Credit Assessment

Overall assessment

Reviewed assessment · Qualified · Recorded outcome: Positive

The management team at HCA Healthcare exhibits strong organizational continuity, evidenced by long-tenured executives and internal promotions, which supports stable governance. The leadership structure incorporates experienced professionals with deep internal institutional knowledge as well as relevant external oversight experience, providing confidence in the leadership succession process.

Material qualifications

- No specific data on recent performance or turnover trends were provided, limiting the ability to assess executive impact on current financial metrics.
- Operational effectiveness depends on ongoing execution, which tenure alone does not guarantee.
- Some submitted supporting evidence was rejected; this finding uses the surviving reviewed evidence.
- Validation withheld 6 evidence claims and 1 judgment; only surviving reviewed findings are presented.

Key credit considerations

Supporting Factors

Leadership Stability and Succession · Supporting Factor · Qualified

Reviewed finding and credit implication

The executive team displays high levels of institutional knowledge, with several senior leaders having multi-decade careers within the company, indicating stable internal succession pipelines and established operating expertise.

[1], [2], [3]

> The saved assessment supports 1 distinct reviewed finding; three to five distinct findings are not supported by the recorded evidence.

Executive records as of 2026-02-01 · 7 sourced executive profiles. Details, evidence gaps and source references follow.

Management Details & Monitoring

Executive roster as of 2026-02-01; recorded transition review window 2023-02-01 to 2026-02-01.

Executive / position	Selected relevant experience	Appointment / transition	Sources
Samuel N. Hazen Chief Executive Officer and Director	President and Chief Operating Officer 2016-11 to 2018-12	2019-01 Outside recent window · Internal succession	[1]
Jennifer L. Berres Senior Vice President and Chief Human Resources Officer	Vice President — Human Resources 2013-04 to 2019-10	2019-11-01 Outside recent window · Internal succession	[1]
Michael S. Cuffe, M.D. Executive Vice President and Chief Clinical Officer	No admitted prior-role detail.	2022-01-01 Outside recent window	[1]
Jon M. Foster Executive Vice President and Chief Operating Officer	President — American Group 2013-01 to 2022-12	2023-01-01 Outside recent window · Internal succession	[1], [2]
Michael A. Marks Executive Vice President and Chief Financial Officer	Senior Vice President — Finance 2023-01 to 2024-04	2024-05-01 Recent appointment · Internal succession	[1], [2]
Michael R. McAlevey Executive Vice President — Chief Legal and Administrative Officer	No admitted prior-role detail.	2024-04-01 Recent appointment	[1], [2]
Erica L. Rossitto Senior Vice President and Chief Nurse Executive	Senior Vice President and Assistant Chief Nurse Executive 2023-12 to 2026-01	2026-02-01 Recent appointment · Internal succession	[1], [3]

> ### Areas Requiring Further Assessment
>
> - Financial Stewardship — Insufficient evidence.
> - Operational Execution Capability — Insufficient evidence.
> - Governance and Integrity — Insufficient evidence.
> - Relevant Expertise — Insufficient evidence.

Follow-up evidence required

- Details on specific management turnover events or recent organizational performance results.

Management monitoring priorities

No management-specific monitoring indicators or thresholds were recorded.

Sources

1. HCA Healthcare, Inc. · Form 10-K · 2025-12-31 · Passage offset 159626
2. HCA Healthcare, Inc. · Form 10-K · 2025-12-31 · Passage offset 161626
3. HCA Healthcare, Inc. · Form 10-K · 2025-12-31 · Passage offset 163626

Complete observations, career histories and review rationale remain in this run's Executive Profiles & Supporting Evidence and Audit & Review.

03 Competitive Positioning

Revenue Drivers

Pricing & reimbursement

Fact | – Negative

Finding	Consolidated buyer behavior and narrow network adoption exert persistent downward pressure on the company's ability to negotiate favorable terms. High. Increasing competitive pressure.
Credit Implication	Reduced ability to leverage market position as payers consolidate and restrict network access. Risk of margin compression if payers successfully force lower reimbursement.
Watch Triggers	Contract renewal outcomes; shifts in share of managed care vs. other payers.

Demand & payer mix

Management statement | – Negative

Finding	Workforce availability and labor costs act as a material constraint on the company's ability to serve potential volume growth. Material. Stable.
Credit Implication	Operational capacity constraints prevent capturing additional demand, limiting revenue growth. Operational risk to growth.
Watch Triggers	Escalation in contract labor usage or service shutdowns.

Management-attributed source finding.

Cross-dimensional insight

Inference | – Negative

Finding	Capacity constraints caused by persistent labor shortages amplify the company's vulnerability to payer-imposed narrow network steering. High Increasing
Credit Implication	Operating capacity limits prevent the company from absorbing volume or optimizing throughput, reducing bargaining leverage when payers restrict network access based on price and site-of-service preference. Reduced resilience to adverse contract negotiations. Operational incapacity facilitates provider exclusion.
Watch Triggers	Divergence between market share growth and industry-wide patient demand.

Qualified source finding. Data correlating local labor vacancy rates with specific contract renewal outcomes. Management-attributed source finding. Specific contract expiration dates or concentration by individual large payer entity.

Cost Drivers

Operating costs

Calculated fact | + Positive

Finding	The company has demonstrated successful containment of workforce costs relative to revenue growth in 2025. Salaries and benefits as a percentage of revenue fell from 44.1% in 2024 to 43.5% in 2025. Favorable improvement in labor cost ratio.
Credit Implication	Operating margin protection via workforce efficiency.

	Cost containment mitigates inflationary pressure on margins. Operating margin
Watch Triggers	Future inability to maintain this ratio if contract labor needs rise.

Revenue & Cost Interactions

Cross-dimensional insight

Inference + Positive

Finding	Enterprise-wide shared services and labor initiatives provide a mitigating cushion against localized inflationary and competitive pressures. Moderate Stable
Credit Implication	Economies of scale and centralized recruitment leverage the network's total size to normalize costs, preventing isolated capacity issues from cascading into enterprise-wide earnings volatility. Stabilizes margins during period-specific disruptions. Scale efficiencies buffering input cost variability.
Watch Triggers	Ability to sustain labor-cost-to-revenue ratios during economic downturns.

Qualified source finding. Direct cost-saving attribution to centralized services. Management-attributed source finding.

Independent challenge was not performed under this workflow. Human review remains required.

Actual patient retention metrics across the network.

Data correlating local labor vacancy rates with specific contract renewal outcomes.

Direct cost-saving attribution to centralized services.

Measured impact on referral volume.

> Measured impact on service volume shifts.

> Quantitative evidence of cost savings specifically attributable to shared services or purchasing scale.

> Segment-level margin disclosure differentiating inpatient from outpatient service lines.

> Specific breakdown separating negotiated rate increases from service mix or volume adjustments within the 'revenue per equivalent admission' metric.

> Specific contract expiration dates or concentration by individual large payer entity.

> Module 1 coverage remains incomplete.

04 Industry

The company is highly dependent on government-funded health programs, which expose the company to regulatory and legislative risks.

Regulatory changes to reimbursement levels and scope of coverage directly impact revenue stability.

Reductions or modifications to Medicare and Medicaid payment programs could materially adversely impact revenue.

45.4% of 2025 revenues were derived from Medicare and Medicaid programs.

Constant regulatory environment monitoring is required due to ongoing policy changes.

Future legislation changing Medicare or Medicaid payment rates or eligibility requirements.

Geographic concentration of facilities creates a heightened exposure to localized external events, particularly in Florida and Texas.

Natural disasters or state-specific regulatory changes can significantly impair a majority of consolidated revenue.

Business continuity and financial results are sensitive to extreme weather events (e.g., hurricanes) in key states.

51% of 2025 consolidated revenues were derived from facilities in Florida and Texas.

Concentration levels remain high; weather risk is a consistent material threat.

A major hurricane or significant change in state healthcare legislation in Florida or Texas.

> preserve qualified narrative qualification/attribution.

Business Overview evidence appendix

Static export of the selected run's supporting records. Full management records remain available in Audit & Review for this run.

Corporate Structure

Analysis run: ar_70066e79dd954e3c · Evidence snapshot: 1

Debt Obligations

Analysis run: ar_70066e79dd954e3c · Evidence snapshot: 1

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> nce and operating leases for the years ended December 31, 2025, 2024 and 2023 (dollars in millions): 2025 2024 2023 Finance lease expense: Depreciation and amortization $ 182 $ 159 $ 164 Interest 41 37 31 Operating leases(1) 516 503 495 Short-term lease expense(1) 317 365 337 Variable lease expense(1) 197 181 162 $ 1,253 $ 1,245 $ 1,189 (1) Expenses are included in “other operating expenses” in our consolidated income statements. The following table presents supplemental cash flow information for the years ended December 31, 2025, 2024 and 2023 (dollars in millions): 2025 2024 2023 Cash paid for amounts included in the measurement of lease liabilities: Operating cash flows for operating leases $ 501 $ 490 $ 479 Operating cash flows for finance leases 41 37 31 Financing cash flows for finance leases 173 172 140 Maturities of Lease Liabilities The following table reconciles the undiscounted minimum lease payment amounts to the operating and finance lease liabilities recorded on the balance sheet at December 31, 2025 and 2024 (dollars in millions): 2025 2024 Operating Leases Finance Leases Operating Leases Finance Leases Year 1 $ 474 $ 173 $ 455 $ 197 Year 2 413 126 405 146 Year 3 343 100 344 99 Year 4 276 85 281 81 Year 5 214 62 221 71 Thereafter 1,499 532 1,460 497 Total minimum lease payments 3,219 1,078 3,166 1,091 Less: amount of lease payments representing interest (1,014) (320) (960) (305) Present value of future minimum lease payments 2,205 758 2,206 786 Less: current lease obligations (352) (133) (343) (162) Long-term lease obligations $ 1,853 $ 625 $ 1,863 $ 624 NOTE 10 — CONTINGENCIES We operate in a highly regulated and litigious industry. As a result, various lawsuits, claims and legal and regulatory proceedings have been and can be expected to be instituted or asserted against us. We are also subject to claims and suits arising in the ordinary course of business, including claims for personal injuries or wrongful restriction of, or interference wi

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> subject to variable rates of interest, while the remaining debt balance of $43.985 billion at December 31, 2025 was subject to fixed rates of interest. Both the general

level of interest rates and, for the senior unsecured credit facility, our leverage affect our variable interest rates. Our variable debt is comprised of outstanding commercial paper notes and the floating rate senior notes due 2028. The average effective interest rate for our long-term debt was 5.0% for both 2025 and 2024. The estimated fair value of our total long-term debt was $45.911 billion at December 31, 2025. The estimates of fair value are based upon the quoted market prices for the same or similar issues of long-term debt with the same maturities. Based on a hypothetical 1% increase in interest rates, the potential annualized reduction to future pretax earnings would be approximately $25 million. To mitigate the impact of fluctuations in interest rates, we generally target a majority of our debt portfolio to be maintained at fixed rates. We are exposed to currency translation risk related to our foreign operations. We currently do not consider the market risk related to foreign currency translation to be material to our consolidated financial statements or our liquidity. Tax Examinations During 2025, the Internal Revenue Service (“IRS”) concluded its examination of the Company’s 2022 and 2023 income tax returns resolving all federal income tax matters for those years. Completion of the examination had no material impact on our results of operations or financial position. At December 31, 2025, the IRS was examining the 2019 income tax returns of certain affiliates of the Company. We are subject to examination by the IRS for tax years after 2023, as well as by state and foreign taxing authorities. Management believes HCA Healthcare, Inc., its subsidiaries and affiliates properly reported taxable income and paid taxes in accordance with applicable laws and agreements established with the IRS,

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of transactions. These covenants limit our and/or certain of our subsidiaries’ ability to, among other things: • incur additional indebtedness or issue certain preferred shares; • create liens; • engage in certain sale and lease-back transactions; and • consolidate, merge, sell or otherwise dispose of all or substantially all of our assets. Under our senior unsecured credit facility, we are required to satisfy and maintain a specified financial ratio. Our ability to maintain this financial ratio may be affected by global economic and financial conditions or other events beyond our control, and there can be no assurance we will continue to maintain this ratio. A breach of this or any other covenant could result in a default under our senior unsecured credit facility, upon which the lenders thereunder could elect to declare all amounts outstanding to be immediately due and payable and terminate all commitments to extend further credit, which would also result in an event of default under a significant portion of our other outstanding 34 indebtedness. In a scenario where the repayment of borrowings is accelerated, there can be no assurance there will be sufficient assets to repay our senior unsecured credit facility and our other indebtedness. Risks related to human capital: Our results of operations may be adversely affected by competition for

staffing, the shortage of experienced nurses and other health care professionals and labor union activity. Our operations are dependent on the efforts, abilities and experience of our management and medical personnel, such as physicians, nurses, pharmacists and lab technicians. We compete with other health care providers in recruiting and retaining qualified management and personnel responsible for the daily operations of each of our hospitals and other facilities, including nurses and other nonphysician health care professionals. We depend on the available labor pool of employees in each of the markets in which we operate to fi

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ncluding $2.207 billion of outstanding commercial paper notes (short-term borrowings). We have the ability to refinance our outstanding commercial paper notes with our senior unsecured credit facility on a long-term basis. Excluding the impact of our outstanding commercial paper notes, our working capital at December 31, 2025 would have been $1.640 billion. Cash used in investing activities was $4.988 billion, $4.933 billion and $5.317 billion in 2025, 2024 and 2023, respectively. Excluding acquisitions, capital expenditures were $4.944 billion in 2025, $4.875 billion in 2024 and $4.744 billion in 2023. Planned capital expenditures are expected to approximate between $5.0 billion and $5.5 billion in 2026. At December 31, 2025, there were projects under construction which had an estimated additional cost to complete and equip over the next five years of approximately $7.1 billion. We expect to fund capital expenditures with internally generated and borrowed funds. We expended $397 million, $266 million and $635 million for acquisitions of hospitals and health care entities during 2025, 2024 and 2023, respectively. Cash flows from sales of hospitals and health care entities were $269 million of net proceeds for 2025, $328 million of net proceeds for 2024 and $193 million of net proceeds for 2023. 69 HCA HEALTHCARE, INC. MANAGEMENT'S DISCUSSION AND ANALYSIS OF FINANCIAL CONDITION AND RESULTS OF OPERATIONS (Continued) Liquidity and Capital Resources (continued) Cash used in financing activities totaled $8.550 billion in 2025, $4.582 billion in 2024 and $4.094 billion in 2023. During 2025, we had a net increase of $3.287 billion in our indebtedness, paid dividends of $679 million and paid $10.067 billion for repurchases of common stock. During 2024, we had a net increase of $3.205 billion in our indebtedness, paid dividends of $690 million and paid $6.042 billion for repurchases of common stock. During 2023, we had a net increase of $1.295 billion in our indebtedness, pa

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uninsured and underinsured patients); potential increased expenses related to labor, pharmaceuticals, supply chain or other expenditures; workforce disruptions; supply and pharmaceutical shortages and disruptions (including as a result of tariffs or geopolitical disruptions); and the impact of federal government shutdowns, holds on or cancellations of congressionally authorized spending and interruptions in the distribution of governmental funds, (2) the impact of current and future health care public policy developments and the implementation of new, and possible changes to existing, federal, state or local laws and regulations affecting the health care industry, including the expiration at the end of 2025 of enhanced premium tax credits ("EPTCs") for eligible individuals purchasing insurance coverage through federal and state-based health insurance marketplaces, changes in the structure and administration of, and funding for, federal and state agencies and programs, and effects of the 2025 Federal Budget Act (the "FBA"), (3) the impact of our significant indebtedness and the ability to refinance such indebtedness on acceptable terms, (4) the effects related to the implementation of sequestration spending reductions required under the Budget Control Act of 2011, related legislation extending these reductions, and the potential for future deficit or other spending reduction legislation that may alter current spending reductions, which include cuts to Medicare payments, or impose additional spending reductions, (5) the ability to achieve operating and financial targets, develop and execute resiliency plans to offset to the extent possible impacts from the FBA, the expiration of EPTCs and tariffs, attain expected levels of patient volumes and revenues, and control the costs of providing services, (6) possible reductions or other changes in Medicare, Medicaid and other state programs, including Medicaid supplemental payment programs, Medicaid waiver programs and state

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e materially and adversely affected. Our business is subject to the following principal risks and uncertainties: Risks related to our indebtedness: • We have significant indebtedness and may incur further indebtedness in the future. Our indebtedness could adversely affect our ability to raise additional capital to fund our operations, limit our ability to 4 react to changes in the economy or our industry, expose us to interest rate risk to the extent of our variable rate debt and prevent us from meeting our obligations. • We may not be able to generate sufficient cash to service all of our indebtedness and may not be able to refinance our indebtedness on favorable terms. If we are unable to do so, we may be forced to take other actions to satisfy our obligations under our indebtedness, which may not be successful. • Our debt agreements contain restrictions that limit our flexibility in operating our business. Risks related to human capital: • Our results of operations may be adversely affected by competition for staffing, the shortage of experienced nurses and other health care professionals and labor union activity. • Our performance depends on our ability to

recruit and retain quality physicians. • We may be unable to attract, hire and retain a highly qualified workforce, including key management. Risks related to technology, data privacy and cybersecurity: • Cybersecurity incidents or other forms of data breaches could result in the compromise of our facilities, confidential data or critical data systems, causing our operations to be impaired or impacted. A cybersecurity incident or other form of data breach could also give rise to potential harm to patients; remediation and other expenses; and exposure to liability under privacy and security laws, consumer protection laws, common law theories or other laws. Such incidents could subject us to litigation and foreign, federal and state governmental inquiries, damage our reputation, and otherwise be disruptive to ou

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sured at fair value on a recurring basis as of December 31, 2025 and 2024, aggregated by the level in the fair value hierarchy within which those measurements fall (dollars in millions): 2025 Fair Value Measurements Using Fair Value Quoted Prices in Active Markets for Identical Assets (Level 1) Significant Other Observable Inputs (Level 2) Significant Unobservable Inputs (Level 3) Debt securities $ 328 $ 1 $ 327 $ — Money market funds and other 260 260 — — Investments of insurance subsidiaries 588 261 327 — Less amounts classified as current assets (103) (103) — — $ 485 $ 158 $ 327 $ — 2024 Fair Value Measurements Using Fair Value Quoted Prices in Active Markets for Identical Assets (Level 1) Significant Other Observable Inputs (Level 2) Significant Unobservable Inputs (Level 3) Debt securities $ 361 $ — $ 361 $ — Money market funds and other 296 296 — — Investments of insurance subsidiaries 657 296 361 — Less amounts classified as current assets (88) (88) — — $ 569 $ 208 $ 361 $ — F-22 HCA HEALTHCARE, INC. NOTES TO CONSOLIDATED FINANCIAL STATEMENTS (Continued) NOTE 7 — ASSETS AND LIABILITIES MEASURED AT FAIR VALUE (continued) The estimated fair value of our long-term debt was $45.911 billion and $40.845 billion at December 31, 2025 and 2024, respectively, compared to carrying amounts, gross of debt issuance costs, premiums and discounts, aggregating $46.928 billion and $43.400 billion, respectively. The estimates of fair value are generally based on Level 2 inputs, including quoted market prices or quoted market prices for similar issues of long-term debt with the same maturities. NOTE 8 — DEBT A summary of our debt at December 31, 2025 and December 31, 2024, including related interest rates at December 31, 2025, follows (dollars in millions): 2025 2024 Short-term borrowings: Commercial paper (average life of 18 days, weighted average rate of 4.3%) $ 2,207 $ — Long-term debt: Senior secured term loan facility — 1,238 Other senior secured debt (effective interes

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tity, and, under certain circumstances, legal and contractual restrictions may limit our ability to obtain cash from our subsidiaries. We may find it necessary or prudent to refinance our outstanding indebtedness, the terms of which refinancing may not be favorable to us. Our ability to refinance our indebtedness on favorable terms, or at all, is directly affected by the then current global economic and financial conditions which affect the availability of debt financing and the rates at which such financing is available. In addition, our ability to incur secured indebtedness depends in part on the value of our assets, which depends, in turn, on the strength of our cash flows and results of operations, and on economic and market conditions and other factors. Downgrades in our credit ratings may also negatively affect availability of debt financing and the rates at which such financing is available. If our cash flows and capital resources are insufficient to fund our debt service obligations or we are unable to refinance our indebtedness, we may be forced to reduce or delay investments and capital expenditures, or to sell assets, seek additional capital or restructure our indebtedness. These alternative measures may not be successful and may not permit us to meet our scheduled debt service obligations. If our operating results and available cash are insufficient to meet our debt service obligations, we could face substantial liquidity problems and might be required to dispose of material assets or operations to meet our debt service and other obligations. We may not be able to consummate those dispositions, or the proceeds from the dispositions may not be adequate to meet any debt service obligations then due. Our debt agreements contain restrictions that limit our flexibility in operating our business. Our senior unsecured credit facility and the indentures governing our outstanding notes contain various covenants that limit our ability to engage in specified types

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ciation (35,134) (33,100) 31,141 29,414 Investments of insurance subsidiaries 485 569 Investments in and advances to affiliates 633 662 Goodwill and other intangible assets 10,293 10,093 Right-of-use operating lease assets 2,130 2,131 Other 255 230 $ 60,720 $ 59,513 LIABILITIES AND STOCKHOLDERS' (DEFICIT) EQUITY Current liabilities: Accounts payable $ 4,659 $ 4,276 Accrued salaries 2,525 2,304 Other accrued expenses 4,277 3,899 Short-term borrowings and long-term debt due within one year 4,889 4,698 16,350 15,177 Long-term debt, less debt issuance costs and discounts of $436 and $369 41,603 38,333 Professional liability risks 1,466 1,544 Right-of-use operating lease obligations 1,853 1,863 Income taxes and other liabilities 2,219 2,041 Stockholders' (deficit) equity: Common stock $0.01 par; authorized 1,800,000,000 shares; outstanding 224,605,100 shares — 2025 and 249,981,400 shares — 2024 2 3 Accumulated other comprehensive loss (305) (387) Retained deficit (5,724) (2,115) Stockholders' deficit attributable to HCA Healthcare, Inc. (6,027) (2,499) Noncontrolling interests 3,256 3,054 (2,771) 555 $ 60,720 $ 59,513 The accompanying

notes are an integral part of the consolidated financial statements. F-7 HCA HEALTHCARE, INC. CONSOLIDATED STATEMENTS OF STOCKHOLDERS' EQUITY (DEFICIT) FOR THE YEARS ENDED DECEMBER 31, 2025, 2024 AND 2023 (Dollars in millions, except per share amounts) Equity (Deficit) Attributable to HCA Healthcare, Inc. Capital Accumulated Equity Common Stock in Excess Other Attributable to Shares Par of Par Comprehensive Retained Noncontrolling (in millions) Value Value Loss Deficit Interests Total Balances, December 31, 2022 277.378 $ 3 $ — $ (490) $ (2,280) $ 2,694 $ (73) Comprehensive income 65 5,242 849 6,156 Repurchase of common stock (14.465) (172) (3,670) (3,842) Share-based benefit plans 2.624 172 172 Cash dividends declared ($2.40 per share) (658) (658) Distributions (640) (640) Other 14 (69) (55) Balances, December 31, 202

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ical volumes declined 0.2% on a consolidated basis and 0.5% on a same facility basis during 2025, compared to 2024. Emergency room visits increased 1.6% on a consolidated basis and 1.8% on a same facility basis during 2025, compared to 2024. The estimated cost of total uncompensated care increased $239 million for 2025, compared to 2024. Consolidated and same facility uninsured admissions increased 1.9% and 1.2%, respectively, and consolidated and same facility uninsured emergency room visits each declined 0.6% for 2025, compared to 2024. Interest expense totaled $2.248 billion for 2025, compared to $2.061 billion for 2024. The $187 million increase in interest expense for 2025 was primarily due to an increase in the average debt balance. Cash flows from operating activities increased $2.122 billion, from $10.514 billion for 2024 to $12.636 billion for 2025. The increase in cash flows from operating activities was related primarily to the combined impact of a $1.319 billion increase in net income, excluding gains on sales of facilities and depreciation and amortization, positive changes in working capital of $524 million and a decline in income taxes paid of $104 million. Business Strategy We are committed to providing the communities we serve with high quality, convenient and cost-effective health care while growing our business and creating long-term value for our stockholders. We strive to be the health care system of choice in the communities we serve by developing comprehensive networks locally and supporting these networks with enterprise expertise and economies of scale. Our strategy is organized around a framework that seeks to drive sustained growth by delivering operational excellence, attracting exceptional physicians and other health care professionals, developing comprehensive services, creating greater access, and coordinating higher quality care for patients. To achieve these objectives, we align our efforts around the following growth agenda: Grow Ou

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,474 7,495 3,224 Net change in short-term borrowings and revolving credit facilities 2,202 (1,880) (1,020) Repayment of long-term debt (7,389) (2,410) (909) Distributions to noncontrolling interests (827) (711) (640) Payment of debt issuance costs (79) (67) (31) Payment of dividends (679) (690) (661) Repurchase of common stock (10,067) (6,042) (3,811) Other (185) (277) (246) Net cash used in financing activities (8,550) (4,582) (4,094) Effect of exchange rate changes on cash and cash equivalents 9 (1) 7 Change in cash and cash equivalents (893) 998 27 Cash and cash equivalents at beginning of period 1,933 935 908 Cash and cash equivalents at end of period $ 1,040 $ 1,933 $ 935 Interest payments $ 2,207 $ 1,938 $ 1,892 Income tax payments, net $ 1,740 $ 1,844 $ 1,386 The accompanying notes are an integral part of the consolidated financial statements. F-9 HCA HEALTHCARE, INC. NOTES TO CONSOLIDATED FINANCIAL STATEMENTS NOTE 1 — ACCOUNTING POLICIES Reporting Entity HCA Healthcare, Inc. is a holding company whose affiliates own and operate hospitals and related health care entities. The term “affiliates” includes direct and indirect subsidiaries of HCA Healthcare, Inc. and partnerships and joint ventures in which such subsidiaries are partners. At December 31, 2025 these affiliates owned and operated 190 hospitals, 121 freestanding surgery centers, 31 freestanding endoscopy centers and provided extensive outpatient and ancillary services. HCA Healthcare, Inc.’s facilities are located in 19 states and England. The terms “Company,” “HCA,” “we,” “our” or “us,” as used herein and unless otherwise stated or indicated by context, refer to HCA Healthcare, Inc. and its affiliates. The terms “facilities” or “hospitals” refer to entities owned and operated by affiliates of HCA and the term “employees” refers to employees of affiliates of HCA. Basis of Presentation The preparation of financial statements in conformity with generally accepted accounting pri

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risk related to changes in interest rates that impact the amount of the interest expense we incur with respect to our floating rate obligations as well as the value of certain investments. We periodically enter into interest rate swap agreements to manage our exposure to these fluctuations. These interest rate swap agreements involve the exchange of fixed and variable rate interest payments between two parties, based on common notional principal amounts and maturity dates. Risks related to ownership of our common stock: There can be no assurance that we will continue to pay dividends. The Company declares a regular quarterly cash dividend under our cash dividend program. During 2025, the Board of Directors declared four quarterly dividends of $0.72 per share, or $2.88 per share in the aggregate, on our common stock. On January 26, 2026, our Board of Directors declared a quarterly

dividend of $0.78 per share on our common stock payable on March 31, 2026 to stockholders of record at the close of business on March 17, 2026. The declaration, amount and timing of such dividends are subject to capital availability and determinations by our Board of Directors that cash dividends are in the best interest of our stockholders and are in compliance with all respective laws and our agreements applicable to the declaration and payment of cash dividends. Our ability to pay dividends will depend upon, among other factors, our cash flows from operations, our available capital and potential future capital requirements for strategic transactions, including acquisitions, debt service requirements, share repurchases and investing in our existing markets as well as our results of operations, financial condition and other factors beyond our control that our Board of Directors may deem relevant. A reduction in or suspension or elimination of our dividend payments could have a negative effect on our stock price. Certain of our investors may continue to have influence over us. On November 1

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Item 1A. Risk Factors If any of the events discussed in the following risk factors were to occur, our business, financial position, results of operations, cash flows or prospects could be materially, adversely affected. Additional risks and uncertainties not presently known to us or that we currently deem immaterial may also affect us. Our business is subject to the following material risks and uncertainties. Risks related to our indebtedness: We have significant indebtedness and may incur further indebtedness in the future. Our indebtedness could adversely affect our ability to raise additional capital to fund our operations, limit our ability to react to changes in the economy or our industry, expose us to interest rate risk to the extent of our variable rate debt and prevent us from meeting our obligations. As of December 31, 2025, our total indebtedness was $46.492 billion. As of December 31, 2025, we had availability of $5.779 billion under our senior unsecured credit facility (after giving effect to all issued and outstanding letters of credit and our intention to maintain a minimum available borrowing capacity equal to the aggregate amount outstanding under the commercial paper program ($2.207 billion as of December 31, 2025)). Our indebtedness could have important consequences, including: • increasing our vulnerability to downturns or adverse changes in general economic, industry or competitive conditions and adverse changes in government regulations; • requiring a portion of cash flows from operations to be dedicated to the payment of principal and interest on our indebtedness, therefore reducing our ability to use our cash flows to fund our operations, capital expenditures and future business opportunities; • exposing us to the risk of increased interest rates on our existing borrowings that are at variable rates of interest or refinancing our debt in a rising or high rate environment; • limiting our ability to make strategic acquisitions or causing us to

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28.33 $ 22.00 $ 18.97 F-20 HCA HEALTHCARE, INC. NOTES TO CONSOLIDATED FINANCIAL STATEMENTS (Continued) NOTE 6 — INVESTMENTS OF INSURANCE SUBSIDIARIES A summary of the insurance subsidiaries' investments at December 31 follows (dollars in millions): 2025 Unrealized Amounts Amortized Cost Gains Losses Fair Value Debt securities $ 342 $ 1 $ (15) $ 328 Money market funds and other 260 — — 260 $ 602 $ 1 $ (15) 588 Amounts classified as current assets (103) Investment carrying value $ 485 2024 Unrealized Amounts Amortized Cost Gains Losses Fair Value Debt securities $ 388 $ — $ (27) $ 361 Money market funds and other 296 — — 296 $ 684 $ — $ (27) 657 Amounts classified as current assets (88) Investment carrying value $ 569 At December 31, 2025 and 2024, the investments in debt securities of our insurance subsidiaries were classified as "available-for-sale." Changes in unrealized gains and losses that are not credit-related are recorded as adjustments to other comprehensive income (loss). Scheduled maturities of investments in debt securities at December 31, 2025 were as follows (dollars in millions): Amortized Cost Fair Value Due in one year or less $ 29 $ 29 Due after one year through five years 145 140 Due after five years through ten years 111 104 Due after ten years 57 55 $ 342 $ 328 The average expected maturity of the investments in debt securities at December 31, 2025 was 3.7 years, compared to the average scheduled maturity of 7.8 years. Expected and scheduled maturities may differ because the issuers of certain securities have the right to call, prepay or otherwise redeem such obligations prior to their scheduled maturity date. F-21 HCA HEALTHCARE, INC. NOTES TO CONSOLIDATED FINANCIAL STATEMENTS (Continued) NOTE 7 — ASSETS AND LIABILITIES MEASURED AT FAIR VALUE Accounting Standards Codification 820, Fair Value Measurements and Disclosures ("ASC 820") emphasizes fair value is a market-based measurement, and fair value measurements should be determined based o

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Florida 98 72 85 All other 157 134 105 Foreign 19 13 9 $ 1,740 $ 1,844 $ 1,386 The provision for income taxes consists of the following (dollars in millions): 2025 2024 2023 Current: Federal $ 1,551 $ 1,202 $ 1,118 State 248 212 213 Foreign 13 20 3 Deferred: Federal 201 394 241 State 31 31 21 Foreign 6 7 19 $ 2,050 $ 1,866 $ 1,615 Our provision for income taxes for the years ended December 31, 2025, 2024 and 2023 included tax benefits of $61 million, $102 million and $93 million, respectively, related to the settlement of employee equity awards. The provision for income taxes reflects a $27 million and $61 million reduction in interest (net of tax) and penalty expense and $36 million of interest expense (net of tax) for the years ended December 31, 2025, 2024 and 2023, respectively. During 2024, we derecognized

deferred tax assets and increased our tax provision by $276 million due to an internal restructuring of certain affiliates. F-18 HCA HEALTHCARE, INC. NOTES TO CONSOLIDATED FINANCIAL STATEMENTS (Continued) NOTE 4 — INCOME TAXES (continued) A reconciliation of the federal statutory rate to the effective income tax rate follows (dollars in millions): 2025 2024 2023 Amount Percent Amount Percent Amount Percent U.S. federal statutory tax rate $ 2,065 21.0 % $ 1,790 21.0 % $ 1,618 21.0 % State and local taxes, net of federal income tax effect (1) 228 2.3 206 2.4 179 2.3 Foreign tax effects (4) — 10 0.1 4 0.1 Nontaxable or nondeductible items Noncontrolling interest (210) (2.1) (188) (2.2) (178) (2.3) Share-based payment awards (55) (0.6) (91) (1.1) (84) (1.1) Other nontaxable or nondeductible items 41 0.5 50 0.6 43 0.6 Changes in unrecognized tax benefits (18) (0.2) (177) (2.1) 33 0.4 Other Adjustments Internal restructuring of affiliates — — 265 3.2 — — Other adjustments, net 3 — 1 — — — Effective tax rate on income before income taxes $ 2,050 20.9 % $ 1,866 21.9 % $ 1,615 21.0 % (1) State taxes in Florida and Texas made up the majority (greater

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3 265.537 3 — (425) (1,352) 2,834 1,060 Comprehensive income 38 5,760 897 6,695 Repurchase of common stock (17.798) (261) (5,803) (6,064) Share-based benefit plans 2.242 261 261 Cash dividends declared ($2.64 per share) (688) (688) Distributions (711) (711) Other (32) 34 2 Balances, December 31, 2024 249.981 3 - (387) (2,115) 3,054 555 Comprehensive income 82 6,784 998 7,864 Repurchase of common stock (26.739) (1) (417) (9,692) (10,110) Share-based benefit plans 1.363 417 417 Cash dividends declared ($2.88 per share) (684) (684) Distributions (827) (827) Other (17) 31 14 Balances, December 31, 2025 224.605 $ 2 $ — $ (305) $ (5,724) $ 3,256 $ (2,771) The accompanying notes are an integral part of the consolidated financial statements. F-8 HCA HEALTHCARE, INC. CONSOLIDATED STATEMENTS OF CASH FLOWS FOR THE YEARS ENDED DECEMBER 31, 2025, 2024 AND 2023 (Dollars in millions) 2025 2024 2023 Cash flows from operating activities: Net income $ 7,782 $ 6,657 $ 6,091 Adjustments to reconcile net income to net cash provided by operating activities: Increase (decrease) in cash from operating assets and liabilities: Accounts receivable (94) (799) (935) Inventories and other assets (154) 334 (126) Accounts payable and accrued expenses 666 359 604 Depreciation and amortization 3,523 3,312 3,077 Income taxes 310 22 229 Losses (gains) on sales of facilities (37) (14) 5 Amortization of debt issuance costs and discounts 44 35 35 Share-based compensation 401 360 262 Other 195 248 189 Net cash provided by operating activities 12,636 10,514 9,431 Cash flows from investing activities: Purchase of property and equipment (4,944) (4,875) (4,744) Acquisition of hospitals and health care entities (397) (266) (635) Sales of hospitals and health care entities 269 328 193 Change in investments 72 (115) (112) Other 12 (5) (19) Net cash used in

investing activities (4,988) (4,933) (5,317) Cash flows from financing activities: Issuances of long-term debt 8

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ein by reference).* 10.36 — HCA Healthcare, Inc. 2025 Executive Officer Performance Excellence Program (filed as Exhibit 10.1 to the Company's Current Report on Form 8-K filed on February 24, 2025, and incorporated herein by reference).* 10.37 — Form of 2025 Stock Appreciation Right Award Agreement under the 2020 Stock Incentive Plan forKey Employees of HCA Healthcare, Inc. and its Affiliates (filed as Exhibit 10.43 to the Company's Annual Report on Form 10-K for the fiscal year ended December 31, 2024, and incorporated herein by reference).* 10.38 — Form of 2025 Performance Share Unit Award Agreement Under the 2020 Stock Incentive Plan for Key Employees of HCA Healthcare, Inc. and its Affiliates (filed as Exhibit 10.44 to the Company's Annual Report on Form 10-K for the fiscal year ended December 31, 2024, and incorporated herein by reference).* 10.39(a) — Credit Agreement dated as of February 20, 2025, by and among HCA Inc., as borrower, Bank of America, N.A., as administrative agent, and the lenders party thereto (filed as Exhibit 10.1 to the Company's Current Report on Form 8-K filed on February 20, 2025, and incorporated herein by reference). 10.39(b) — Amendment No. 1 to Credit Agreement, dated as of October 23, 2025, by and among HCA Inc., as borrower, Bank of America, N.A., as administrative agent, and the lenders party thereto (filed as Exhibit 10.1 to the Company's Quarterly Report on Form 10-Q for the quarter ended September 30, 2025, and incorporated herein by reference). 86 10.40 — Form of 2026 Stock Appreciation Right Award Agreement under the 2020 Stock Incentive Plan for Key Employees of HCA Healthcare, Inc. and its Affiliates.* 10.41 — Form of 2026 Performance Share Unit Award Agreement Under the 2020 Stock Incentive Plan for Key Employees of HCA Healthcare, Inc. and its Affiliates.* 19 — HCA Healthcare, Inc. Securities Trading Policy. 21 — List of Subsidiaries. 22 — List of Subsidiary Guarantors and Pledged Securities. 23 — Consent of Ernst & Young

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October 31, 2025, among HCA Inc., HCA Healthcare, Inc., CSC Delaware Trust Company, as trustee, and Deutsche Bank Trust Company Americas, as paying agent, registrar and transfer agent (filed as Exhibit 4.2 to the Company's Current Report on Form 8-K filed on October 31, 2025, and incorporated herein by reference). 4.84 — Supplemental Indenture No. 51, dated as of October 31, 2025, among HCA Inc., HCA Healthcare, Inc., CSC Delaware Trust Company, as trustee, and Deutsche Bank Trust Company Americas, as paying agent, registrar and transfer agent (filed as Exhibit 4.3

to the Company's Current Report on Form 8-K filed on October 31, 2025, and incorporated herein by reference). 4.85 — Supplemental Indenture No. 52, dated as of October 31, 2025, among HCA Inc., HCA Healthcare, Inc., CSC Delaware Trust Company, as trustee, and Deutsche Bank Trust Company Americas, as paying agent, registrar and transfer agent (filed as Exhibit 4.4 to the Company's Current Report on Form 8-K filed on October 31, 2025, and incorporated herein by reference). 4.86 — Supplemental Indenture No. 53, dated as of October 31, 2025, among HCA Inc., HCA Healthcare, Inc., CSC Delaware Trust Company, as trustee, and Deutsche Bank Trust Company Americas, as paying agent, registrar and transfer agent (filed as Exhibit 4.5 to the Company's Current Report on Form 8-K filed on October 31, 2025, and incorporated herein by reference). 4.87 — Form of 4.300% Senior Notes due 2030 (included in Exhibit 4.83). 4.88 — Form of 4.600% Senior Notes due 2032 (included in Exhibit 4.84). 4.89 — Form of 4.900% Senior Notes due 2035 (included in Exhibit 4.85). 4.90 — Form of 5.700% Senior Notes due 2055 (included in Exhibit 4.86). 10.1 — Form of Indemnity Agreement with certain officers and directors (filed as Exhibit 10.3 to the Company's Registration Statement on Form S-4 (File No. 333-145054) and incorporated herein by reference). 10.2(a) — 2006 Stock Incentive Plan for Key Employees of HCA Holdings, Inc. and its Affili

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of the SERP and our other benefit plans. Benefits expense under the plan was $8 million for 2025, $7 million for 2024 and $10 million for 2023. Accrued benefits liabilities under this plan totaled $126 million at December 31, 2025 and $109 million at December 31, 2024. We maintain defined benefit pension plans which resulted from certain hospital acquisitions in prior years. The amount recognized under these plans was a $7 million credit for 2025, $6 million credit for 2024 and $2 million expense for 2023. Net assets available for benefits in excess of the projected benefit obligation under these plans were $165 million and $118 million at December 31, 2025 and 2024, respectively. NOTE 13 — SEGMENT AND GEOGRAPHIC INFORMATION We operate in one line of business, which is operating hospitals and related health care entities. We operate in three geographically organized groups: the National, Atlantic and American Groups. At December 31, 2025, the National Group included 53 hospitals located in Alaska, California, Idaho, Kentucky, Nevada, New Hampshire, North Carolina, Tennessee, Utah and Virginia; the Atlantic Group included 63 hospitals located in Florida, Georgia, Northern Kansas, Missouri and South Carolina; and the American Group included 66 hospitals located in Colorado, Central Kansas, Louisiana and Texas. The eight hospitals we operate in England are included in the Corporate and other group. F-27 HCA HEALTHCARE, INC. NOTES TO CONSOLIDATED FINANCIAL STATEMENTS (Continued) NOTE 13 — SEGMENT AND GEOGRAPHIC INFORMATION (continued) Adjusted segment EBITDA is defined as

income before depreciation and amortization, interest expense, losses and gains on sales of facilities, losses on retirement of debt, income taxes and net income attributable to noncontrolling interests. We use adjusted segment EBITDA as an analytical indicator for purposes of allocating resources to geographic areas and assessing their performance. Adjusted segment EBITDA is commonly used as an analyt

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t rate of 4.6%) 1,021 1,046 Senior unsecured credit facilities — — Senior unsecured notes payable through 2095 (effective interest rate of 5.1%) 43,700 41,116 Debt issuance costs and discounts (436) (369) Total long-term debt (average life of 11.9 years, rates averaging 5.1%) 44,285 43,031 Total debt 46,492 43,031 Less amounts due within one year 4,889 4,698 $ 41,603 $ 38,333 2025 Financing Activities We issued $5.250 billion aggregate principal amount of senior notes comprised of (i) $700 million aggregate principal amount of 5.000% senior notes due 2028, (ii) $300 million aggregate principal amount of floating rate senior notes due 2028, (iii) $750 million aggregate principal amount of 5.250% senior notes due 2030, (iv) $750 million aggregate principal amount of 5.500% senior notes due 2032, (v) $1.500 billion aggregate principal amount of 5.750% senior notes due 2035 and (vi) $1.250 billion aggregate principal amount of 6.200% senior notes due 2055. We used the net proceeds to repay borrowings under the senior unsecured credit facility and for general corporate purposes. We also issued $3.250 billion aggregate principal amount of senior notes comprised of (i) $500 million aggregate principal amount of 4.300% senior notes due 2030, (ii) $1.000 billion aggregate principal amount of 4.600% senior notes due 2032, (iii) $1.000 billion aggregate principal amount of 4.900% senior notes due 2035 and (iv) $750 million aggregate principal amount of 5.700% senior notes due 2055. We used the net proceeds to repay borrowings under the commercial paper program and for general corporate purposes. We repaid at maturity all $2.600 billion aggregate principal amount of 5.375% senior notes, all $1.400 billion aggregate principal amount of 5.25% senior notes, $291 million aggregate principal amount of 7.69% senior notes and $125 million aggregate principal amount of 7.58% medium-term notes. We also redeemed all $1.500 billion aggregate principal amount of 5.875% senior notes due 2

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certain of these expenses and expect inflationary pressures will continue to impact operating expenses in 2026. Equity in earnings of affiliates was $78 million for 2025 and $23 million for 2024. Depreciation and amortization, as a percentage of revenues,

were 4.6% in 2025 and 4.7% in 2024. Depreciation expense was $3.508 billion for 2025 and $3.294 billion for 2024. The increase of $214 million in depreciation expense relates primarily to capital expenditures at our existing facilities. Interest expense increased to $2.248 billion for 2025 from $2.061 billion for 2024. The $187 million increase in interest expense was primarily due to an increase in the average debt balance. The average effective interest rate for our long-term debt was 5.0% for both 2025 and 2024. Our average debt balance was $44.731 billion for 2025 compared to $41.388 billion for 2024. Gains on sales of facilities were $37 million for 2025 and $14 million for 2024. 68 HCA HEALTHCARE, INC. MANAGEMENT'S DISCUSSION AND ANALYSIS OF FINANCIAL CONDITION AND RESULTS OF OPERATIONS (Continued) Results of Operations (continued) Years Ended December 31, 2025 and 2024 (continued) The effective income tax rate was 23.2% for 2025 and 24.5% for 2024, excluding net income attributable to noncontrolling interests as it relates to consolidated partnerships. The decline in the effective tax rate for 2025 is due to a net increase in our 2024 tax provision related to an internal restructuring of certain affiliates and adjustments to our liability for unrecognized tax benefits. Our provisions for income taxes for 2025 and 2024 included tax benefits of $61 million and $102 million, respectively, related to employee equity award settlements. Net income attributable to noncontrolling interests increased from $897 million for 2024 to $998 million for 2025. The increase in net income attributable to noncontrolling interests related primarily to the operations of two of our Texas markets. For results of operations comparis

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lated to the effects of trends in average severity and average frequency of claims. F-3 How We Addressed the Matter in Our Audit We tested management's internal controls that address the risks of material misstatement over the Company's reserves for professional liability risks estimation process. For example, we tested internal controls over management's review of the actuarial analyses, the significant assumptions, and the completeness and accuracy of claims data used in the reserve estimation process. To test the Company's determination of the estimated professional liability expense and reserves, we performed audit procedures that included, among others, testing the completeness and accuracy of underlying claims data used by the Company and its actuaries in its determination of reserves and reviewing the Company's insurance contracts to validate self-insured limits, deductibles and coverage limits. Additionally, with the involvement of our actuarial specialists, we performed audit procedures that included, among others, assessing the actuarial analyses performed by management and its actuaries, testing the significant assumptions including consideration of Company-specific claim reporting and payment data, assessing the accuracy of management's historical reserve estimates,

and developing an independent range of reserves for comparison to the Company's recorded amounts. /s/ Ernst & Young LLP We have served as the Company's auditor since 1994. Nashville, Tennessee February 10, 2026 F-4 HCA HEALTHCARE, INC. CONSOLIDATED INCOME STATEMENTS FOR THE YEARS ENDED DECEMBER 31, 2025, 2024 AND 2023 (Dollars in millions, except per share amounts) 2025 2024 2023 Revenues $ 75,600 $ 70,603 $ 64,968 Salaries and benefits 32,859 31,170 29,487 Supplies 11,367 10,755 9,902 Other operating expenses 15,886 14,819 12,875 Equity in earnings of affiliates (78) (23) (22) Depreciation and amortization 3,523 3,312 3,077 Interest expense 2,248 2,061 1,938 Losses (gains) on sales of fac

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formance and trends between reporting periods. Information on how these metrics and statistical information are defined is provided in the following tables summarizing operating results and operating data. 65 HCA HEALTHCARE, INC. MANAGEMENT'S DISCUSSION AND ANALYSIS OF FINANCIAL CONDITION AND RESULTS OF OPERATIONS (Continued) Results of Operations (continued) Operating Results Summary The following are comparative summaries of operating results and certain operating data for the years ended December 31, 2025, 2024 and 2023 (dollars in millions): 2025 2024 2023 Amount Ratio Amount Ratio Amount Ratio Revenues $ 75,600 100.0 $ 70,603 100.0 $ 64,968 100.0 Salaries and benefits 32,859 43.5 31,170 44.1 29,487 45.4 Supplies 11,367 15.0 10,755 15.2 9,902 15.2 Other operating expenses 15,886 21.0 14,819 21.0 12,875 19.8 Equity in earnings of affiliates (78) (0.1) (23) — (22) — Depreciation and amortization 3,523 4.6 3,312 4.7 3,077 4.7 Interest expense 2,248 3.0 2,061 2.9 1,938 3.0 Losses (gains) on sales of facilities (37) — (14) — 5 — 65,768 87.0 62,080 87.9 57,262 88.1 Income before income taxes 9,832 13.0 8,523 12.1 7,706 11.9 Provision for income taxes 2,050 2.7 1,866 2.7 1,615 2.5 Net income 7,782 10.3 6,657 9.4 6,091 9.4 Net income attributable to noncontrolling interests 998 1.3 897 1.2 849 1.3 Net income attributable to HCA Healthcare, Inc. $ 6,784 9.0 $ 5,760 8.2 $ 5,242 8.1 % changes from prior year: Revenues 7.1 % 8.7 % 7.9 % Income before income taxes 15.4 10.6 (10.2) Net income attributable to HCA Healthcare, Inc. 17.8 9.9 (7.1) Admissions(a) 2.7 5.0 2.7 Equivalent admissions(b) 2.9 5.3 4.9 Revenue per equivalent admission 4.0 3.2 2.8 Same facility % changes from prior year(c): Revenues 6.6 7.9 7.6 Admissions(a) 2.3 4.9 3.3 Equivalent admissions(b) 2.4 4.5 4.8 Revenue per equivalent admission 4.1 3.2 2.7 (a) Represents the total number of patients admitted to our hospitals and is used by management and certain investors as a general measure of inpatient

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e Year Ended December 31, 2023 National Group Atlantic Group American Group Revenues $ 18,105 $ 21,167 $ 22,318 Salaries and benefits 7,196 8,058 8,080 Supplies 2,658 3,331 3,616 Other operating expenses 4,253 5,289 5,473 Equity in (earnings) losses of affiliates (2) (3) (59) 14,105 16,675 17,110 Adjusted segment EBITDA $ 4,000 $ 4,492 $ 5,208 For the Year Ended December 31, 2025 2024 2023 Adjusted segment EBITDA: National Group $ 5,103 $ 4,392 $ 4,000 Atlantic Group 5,642 4,938 4,492 American Group 6,296 5,729 5,208 17,041 15,059 13,700 Adjustments to reconcile Total Adjusted segment EBITDA to consolidated Income before income taxes: Corporate and Other 1,475 1,177 974 Depreciation and amortization 3,523 3,312 3,077 Interest expense 2,248 2,061 1,938 Losses (gains) on sales of facilities (37) (14) 5 Income before income taxes $ 9,832 $ 8,523 $ 7,706 For the Year Ended December 31, 2025 2024 2023 Revenues: National Group $ 21,278 $ 19,656 $ 18,105 Atlantic Group 24,709 23,380 21,167 American Group 26,445 24,668 22,318 Corporate and other 3,168 2,899 3,378 $ 75,600 $ 70,603 $ 64,968 Depreciation and amortization: National Group $ 905 $ 857 $ 834 Atlantic Group 1,122 1,061 989 American Group 1,131 1,083 971 Corporate and other 365 311 283 $ 3,523 $ 3,312 $ 3,077 F-29 HCA HEALTHCARE, INC. NOTES TO CONSOLIDATED FINANCIAL STATEMENTS (Continued) NOTE 13 — SEGMENT AND GEOGRAPHIC INFORMATION (continued) December 31, 2025 2024 2023 Assets: National Group $ 13,596 $ 12,855 $ 12,487 Atlantic Group 17,945 17,168 16,098 American Group 21,217 20,714 19,786 Corporate and other 7,962 8,776 7,840 $ 60,720 $ 59,513 $ 56,211 National Group Atlantic Group American Group Corporate and Other Total Goodwill and other intangible assets: Balance at December 31, 2022 $ 1,244 $ 2,057 $ 5,152 $ 1,200 $ 9,653 Acquisitions — 8 326 28 362 Foreign currency translation, amortization and other (3) (1) — (66) (70) Balance at December 31, 2023 1,241 2,064 5,478 1,162 9,945 Acquisitions — 61 1

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ely. Our interest expense was $2.248 billion for 2025 and $2.061 billion for 2024. 70 HCA HEALTHCARE, INC. MANAGEMENT'S DISCUSSION AND ANALYSIS OF FINANCIAL CONDITION AND RESULTS OF OPERATIONS (Continued) Liquidity and Capital Resources (continued) Financing Activities (continued) During 2025, we entered into a new credit agreement that provides for $8.000 billion of senior unsecured revolving credit commitments with a term of five years ("senior unsecured credit facility"). Borrowings under the senior unsecured credit facility bear interest at a rate equal to the Secured Overnight Financing Rate plus 1.125% (plus, until October 23, 2025, a 0.10% credit spread adjustment, as the unsecured credit facility was amended on that date to remove the credit spread adjustment). We terminated our $4.500 billion senior secured asset-based revolving credit facility, our $3.500 billion senior secured revolving cash flow credit facility and our senior secured term loan facility of $1.238

billion. Finance leases and other secured debt totaled $1.021 billion at December 31, 2025. During 2025, we issued $5.250 billion aggregate principal amount of senior notes comprised of (i) $700 million aggregate principal amount of 5.000% senior notes due 2028, (ii) $300 million aggregate principal amount of floating rate senior notes due 2028, (iii) $750 million aggregate principal amount of 5.250% senior notes due 2030, (iv) $750 million aggregate principal amount of 5.500% senior notes due 2032, (v) $1.500 billion aggregate principal amount of 5.750% senior notes due 2035 and (vi) $1.250 billion aggregate principal amount of 6.200% senior notes due 2055. We used the net proceeds to repay borrowings under the senior unsecured credit facility and for general corporate purposes. During 2025, we also issued $3.250 billion aggregate principal amount of senior notes comprised of (i) $500 million aggregate principal amount of 4.300% senior notes due 2030, (ii) $1.000 billion aggregate principal amou

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nt of 4.600% senior notes due 2032, (iii) $1.000 billion aggregate principal amount of 4.900% senior notes due 2035 and (iv) $750 million aggregate principal amount of 5.700% senior notes due 2055. We used the net proceeds to repay borrowings under the commercial paper program and for general corporate purposes. During 2025, we established a commercial paper program under which we may issue unsecured commercial paper notes from time to time up to a maximum aggregate face or principal amount of $4.000 billion outstanding at any time. Amounts available under the program may be borrowed, repaid and reborrowed from time to time. The maturities of the commercial paper notes borrowings may vary, but will not exceed 397 days from the date of issue, and the proceeds from the program will be used for general corporate purposes. In connection with the commercial paper program, we intend to maintain a minimum available borrowing capacity under our $8.000 billion senior unsecured credit facility equal to the aggregate amount outstanding under the commercial paper program. At December 31, 2025, we had $2.207 billion of commercial paper outstanding, and there were no borrowings outstanding under our senior unsecured credit facility. During 2025, we repaid at maturity all $2.600 billion aggregate principal amount of 5.375% senior notes, all $1.400 billion aggregate principal amount of 5.25% senior notes, $291 million aggregate principal amount of 7.69% senior notes and $125 million aggregate principal amount of 7.58% medium-term notes. We also redeemed all $1.500 billion aggregate principal amount of 5.875% senior notes due 2026. Management believes that cash flows from operations, amounts available under our senior unsecured credit facility and our anticipated access to public and private debt markets will be sufficient to meet expected liquidity needs for the foreseeable future. All of the senior notes issued by HCA Inc. in 2014 or later are fully and unconditionally guaranteed

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make nonstrategic divestitures; 33 • limiting our ability to obtain additional financing for working capital, capital expenditures, share repurchases, dividends, product or service line development, debt service requirements, acquisitions and general corporate or other purposes; and • limiting our ability to adjust to changing market conditions and placing us at a competitive disadvantage compared to our competitors who have less debt. We and our subsidiaries have the ability to incur additional indebtedness in the future, subject to the restrictions contained in our senior unsecured credit facility and the indentures governing our outstanding notes. If new indebtedness is added to our current debt levels, interest rates and the related risks that we now face could intensify. We may not be able to generate sufficient cash to service all of our indebtedness and may not be able to refinance our indebtedness on favorable terms. If we are unable to do so, we may be forced to take other actions to satisfy our obligations under our indebtedness, which may not be successful. Our ability to make scheduled payments on or to refinance our debt obligations depends on our financial condition and operating performance, which are subject to prevailing economic and competitive conditions and to certain financial, business and other factors beyond our control. We cannot guarantee we will maintain a level of cash flows from operating activities sufficient to permit us to pay the principal, premium, if any, and interest on our indebtedness. In addition, we conduct our operations through our subsidiaries. Accordingly, repayment of our indebtedness is dependent on the generation of cash flows by our subsidiaries and their ability to make such cash available to us by dividend, debt repayment or otherwise. Our subsidiaries may not be able to, or may not be permitted to, make distributions to enable us to make payments in respect of our indebtedness. Each subsidiary is a distinct legal en

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026. Senior Unsecured Credit Facility And Other Senior Secured Debt During 2025, we entered into a new credit agreement that provides for $8.000 billion of senior unsecured revolving credit commitments with a term of five years (“senior unsecured credit facility”). Borrowings under the senior unsecured credit facility bear interest at a rate equal to the Secured Overnight Financing Rate plus 1.125% (plus, until October 23, 2025, a 0.10% credit spread adjustment, as the unsecured credit facility was amended on that date to remove the credit spread adjustment). We terminated our $4.500 billion senior secured asset-based revolving credit facility, our $3.500 billion senior secured revolving cash flow credit facility and our senior secured term loan facility of $1.238 billion. Finance leases and other secured debt totaled $1.021 billion at December 31, 2025. F-23 HCA HEALTHCARE, INC. NOTES TO CONSOLIDATED

FINANCIAL STATEMENTS (Continued) NOTE 8 — DEBT (continued) Commercial Paper Program During 2025, we established a commercial paper program under which we may issue unsecured commercial paper notes from time to time up to a maximum aggregate face or principal amount of $4.000 billion outstanding at any time. Amounts available under the program may be borrowed, repaid and reborrowed from time to time. The maturities of the commercial paper notes borrowings may vary, but will not exceed 397 days from the date of issue, and the proceeds from the program will be used for general corporate purposes. In connection with the commercial paper program, we intend to maintain a minimum available borrowing capacity under our $8.000 billion senior unsecured credit facility equal to the aggregate amount outstanding under the commercial paper program. At December 31, 2025, we had $2.207 billion of commercial paper outstanding, and there were no borrowings outstanding under our senior unsecured credit facility. Senior Unsecured Notes Senior unsecured notes consist of (i) $43.250 billion

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by our Board of Directors that cash dividends are in the best interest of stockholders and are in compliance with all applicable laws and agreements of the Company. In addition to cash flows from operations, available sources of capital include amounts available under our senior unsecured credit facility ($5.779 billion and $5.664 billion available as of December 31, 2025 and January 31, 2026, respectively, after giving effect to all issued and outstanding letters of credit and our intention to maintain a minimum available borrowing capacity equal to the aggregate amount outstanding under the commercial paper program ($2.207 billion and $2.322 billion as of December 31, 2025 and January 31, 2026, respectively) and anticipated access to public and private debt markets. Investments of our insurance subsidiaries, held to maintain statutory equity levels and to provide liquidity to pay claims, totaled $588 million and $657 million at December 31, 2025 and 2024, respectively. An insurance subsidiary maintained net reserves for professional liability risks of $91 million and $127 million at December 31, 2025 and 2024, respectively. Our facilities are insured by one of our insurance subsidiaries for losses up to $110 million per occurrence ($120 million effective January 1, 2026); however, this coverage is subject, in most cases, to a $15 million per occurrence self-insured retention. Net reserves for the self-insured professional liability risks retained were $1.906 billion and $1.924 billion at December 31, 2025 and 2024, respectively. Claims payments, net of reinsurance recoveries, during the next 12 months are expected to approximate $568 million. We estimate that approximately $524 million of the expected net claim payments during the next 12 months will relate to claims subject to the self-insured retention. Financing Activities We have significant debt service requirements. Our debt totaled $46.492 billion and $43.031 billion at December 31, 2025 and 2024, respectiv

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Competitive Positioning

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> ll other necessary positions. In some markets, the availability of nonphysician health care professionals and medical support personnel has been a significant operating issue to health care providers, including at certain of our facilities. The impact of labor shortages across the health care industry may result in other health care facilities, such as nursing homes, limiting admissions, which may constrain our ability to discharge patients to such facilities, increase labor costs and further exacerbate the demand on our resources, supplies and staffing. Economic conditions, including macroeconomic uncertainties and inflationary pressure, workforce burnout, and public health conditions and other factors, have exacerbated workforce competition, personnel shortages and capacity constraints. New limitations on federal loan eligibility, other student loan changes imposed pursuant to the FBA and changes to immigration policies may also impact health care personnel shortages. We may be required to increase wages and benefits to recruit and retain nurses and other medical support personnel and to hire more expensive temporary or contract personnel. If there is additional union organizing activity or a significant portion of our employee base unionizes, it is possible our labor costs could increase. When negotiating collective bargaining agreements with unions, whether such agreements are renewals or first contracts, we have experienced, and could experience in the future, labor strikes. Our continued operation during any strikes could result in an increase to our labor costs. In addition, upon the expiration of existing collective bargaining agreements, we may not reach new agreements without union action, and any such new agreements may not be on terms satisfactory to us. The unavailability of staff, or the inability of the Company to control labor costs, could have a material, adverse effect on our capacity, growth prospects and results of operations. In addition, federa

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s and obtain discounts from providers' established gross charges. Similarly, employers and traditional health insurers continue to attempt to contain costs through negotiations with providers for managed care programs and discounts from established gross charges. Generally, hospitals compete for service contracts with group purchasers of health care services on the basis of price, market reputation, geographic location, quality and range of services, quality of the medical staff and convenience. Legislative and regulatory initiatives may impact our contract terms or ability to contract with payers, such as laws that permit payers to guide patients to particular providers and eliminate restrictions on placing providers into preferred tiers. Our future success will depend, in part, on our ability to retain and renew our contracts with third-party payers and enter into new contracts on favorable terms. Other health care providers may impact our ability to enter into contracts with third-party payers or negotiate increases in our reimbursement and other favorable terms and conditions. For example, some of our competitors may negotiate exclusivity provisions with managed care plans or otherwise restrict the ability of managed care companies to contract with us. Price transparency initiatives and increasing vertical integration efforts involving third-party payers and health care providers, among other factors, may increase these challenges. Moreover, the trend toward consolidation among private third-party payers tends to increase payer bargaining power over fee structures, and private third-party payers may increasingly demand reduced fees or be unwilling to negotiate reimbursement increases. Health plans increasingly utilize narrow networks that restrict the number of participating providers or tiered networks that impose significantly higher cost sharing obligations on patients who obtain services from providers in a disfavored tier. The importance of obtaining contra

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Operating costs

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valent admission compared to 2024. Same facility revenues increased 6.6% due primarily to the combined impact of a 2.4% increase in equivalent admissions and a 4.1% increase in revenue per equivalent admission compared to 2024. Our revenues from Medicaid state directed and supplemental payment programs totaled approximately $6.2 billion and $5.5 billion in 2025 and 2024, respectively. Salaries and benefits, as a percentage of revenues, were 43.5% in 2025 and 44.1% in 2024. Salaries and benefits per equivalent admission increased 2.4% in 2025 compared to 2024. Same facility salaries and benefits per full time equivalent increased 3.3% for 2025 compared to 2024. We continue to utilize certain contract, overtime and other premium rate labor costs to support our clinical staff and patients. While these labor

costs have declined compared to the prior year period, future costs may be affected by labor market conditions and other factors. Share-based compensation expense was $401 million in 2025 and $360 million in 2024. Supplies, as a percentage of revenues, were 15.0% in 2025 and 15.2% in 2024. Supply costs per equivalent admission increased 2.7% in 2025 compared to 2024. Supply costs per equivalent admission increased 7.0% for medical devices and 0.3% for general medical and surgical items, but declined 4.0% for pharmacy supplies in 2025 compared to 2024. The increase in supply costs per equivalent admission for medical devices is primarily related to cardiovascular technologies. The decline in supply costs per equivalent admission for pharmacy supplies is primarily related to a decrease in the costs of certain drugs. Other operating expenses, as a percentage of revenues, were 21.0% in both 2025 and 2024. Other operating expenses are primarily comprised of contract services, professional fees, repairs and maintenance, rents and leases, utilities, insurance (including professional liability insurance) and nonincome taxes. We have seen inflation have a negative impact on

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Cross-dimensional insight

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oviders and other team members who serve at our facilities. We believe a critical component of our growth strategy is our ability to successfully recruit and strategically collaborate with physicians and other health care professionals to provide high quality care. We attract and retain physicians and other health care professionals by providing high quality, convenient facilities with advanced technology, by expanding our specialty services and by building our outpatient operations. We believe our continued investment in the employment, recruitment and retention of physicians and other health care professionals will improve the quality of care at our facilities. Continue to Utilize Economies of Scale to Grow the Company. We believe there is significant opportunity to continue to grow our company by fully utilizing the scale and scope of our organization. We continue to invest in initiatives such as care navigators, clinical data exchange and centralized patient transfer operations, which will enable us to improve coordination of care and patient retention across our markets. We believe our centrally managed business processes and ability to leverage cost-saving practices across our extensive network will enable us to continue to manage costs effectively. We continue to invest in our shared service platforms to deploy key components of our support infrastructure, including revenue cycle management, health care group purchasing, supply chain management and staffing functions. Pursue a Disciplined Development Strategy. We continue to believe there are significant growth opportunities in our markets. We will continue to provide financial and operational resources to analyze and develop our in-market opportunities. To complement our in-market growth agenda and achieve cost savings and other benefits for the patients and communities we serve, we intend to focus on selectively developing and acquiring new hospitals, outpatient facilities and other health care service prov

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Pricing & reimbursement

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rs may utilize plan structures such as narrow networks and tiered networks that limit beneficiary provider choices, impose significantly higher cost-sharing obligations when care is obtained from providers in a disfavored tier or otherwise shift greater financial responsibility for care to individuals. Legislative and regulatory initiatives may accelerate or otherwise impact these trends. Cost-reduction strategies by large employer groups and their affiliates, such as directly contracting with a limited number of providers, may also limit our ability to negotiate favorable terms in our contracts and otherwise intensify competitive pressure. Our ability to retain and renew our third-party payer contracts and enter into new contracts on terms favorable to us may be impacted by other health care providers. For example, some of our competitors may negotiate exclusivity provisions with managed care plans or otherwise restrict the ability of managed care plans to contract with us. Further, shifts in the payer contracts held by our competitors may impact our patient mix, which could negatively impact our revenues. Trends toward greater price transparency may also negatively impact our ability to negotiate favorable contracts with payers. For example, hospitals are required to publish online payer-specific and de-identified minimum and maximum negotiated charges. In addition, health insurers are required to provide online price comparison tools to help individuals get personalized cost estimates for covered items and services. In addition, alignment efforts between third-party payers and health care providers and transparency requirements provide payers with increased access to performance and pricing data, which may increase payer bargaining power. If we are unable to retain and negotiate favorable contracts with third-party payers or experience reductions in payment increases or amounts received from third-party payers or the number of patients with private health insuran

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Demand & payer mix

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